MAHATMA GANDHI UNIVERSITY, KOTTAYAM

MGU-UGP (HONOURS) REGULAR EXAMINATION OCTOBER 2025
THIRD SEMESTER

DISCIPLINE SPECIFIC ELECTIVE (DSE)-MG3DSEECO205
ECONOMICS OF HEALTH AND EDUCATION
(2024 ADMISSION ONWARDS)

SCHEME OF VALUATION

Duration: 2 Hours **Maximum Marks:70**

Part A

1. b). Incidence or prevalence of disease/illness

2. a). Quantity of life (life expectancy) and quality of life (health status)

3. b). High out-of-pocket expenditure (OOPE) on healthcare.

4. b). To achieve universal access to good quality healthcare services without financial hardship

5. b). Unequal access, quality gaps and limited research output compared to top global universities

6. b. gain

7. b). Students' learning environment and outcomes

8. b) The migration of highly skilled professionals to other countries

9. d). Maternal education and neonatal mortality rate

10. c) Higher dependency ratio and need for elderly support systems

Part B

11. Income elastic demand means demand changes with the changes in income-Example — preference for private hospital care in Kerala- When income rises, people prefer private hospitals instead of public ones-Health services are seen as a superior good-Shows positive relation between income and healthcare demand

12. Poverty–malnutrition trap means a cycle linking low income and poor health -Poor people cannot afford enough nutritious food - Leads to low productivity and poor health-Low

productivity reduces income again-The cycle of poverty and malnutrition continues across generations.

13. NRHM started in 2005 to improve rural health care-Aim — provide accessible, affordable, and quality health services-Reduce maternal and infant mortality-Strengthen public health infrastructure-Promote community participation through ASHA workers-Focus on primary health care and rural poor.

14. Institutional discrimination limits equal access to healthcare-Marginalized groups face neglect and bias in hospitals- Leads to poor health outcomes and higher disease rates-Weak implementation of inclusive health policies -Reduces trust in public health systems-Deepens social and economic inequality.

15. Introduce online appointment and queue system - Increase number of doctors and nurses-Use patient feedback for service improvement-Apply time-based performance monitoring -Create separate counters for emergency and routine cases-Promote staff training for personal care and empathy

16. Infant Mortality Rate (IMR) – number of deaths of infants under one year per 1,000 live births- Maternal Mortality Ratio (MMR) – number of maternal deaths per 100,000 live births-Both are key mortality indicators used in National Family Health Survey (NFHS)

17. NHM plays a key role in improving public health services-Strengthens PHCs, CHCs, and Sub-Centres in rural areas-Promotes maternal and child health programs -Increases community participation through ASHA workers-Improves access, quality, and equity in healthcare-Reduces regional health disparities.

18. Janani Suraksha Yojana (JSY) – promotes institutional deliveries-National Immunization Program – tracks child vaccination coverage-Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA) – monitors maternal health check-ups-National AIDS Control Program (NACP) – evaluates HIV awareness and prevention- All the public health initiatives use NFHS data to assess impact and improve policies - For Example, Janani Suraksha Yojana (JSY) promotes institutional deliveries. Evaluated using NFHS data on maternal and child health- Tracks number of women receiving cash Incentives-Assesses impact on maternal and infant mortality rates- Helps improve public health policy planning.

19. Private benefits – advantages to individual like higher salary, better job- Example – a student earns more after college.-Social benefits – advantages to society like skilled workforce, lower crime. Example – educated people improve community health and productivity.

20. Health is key part of human capital-Healthy people are more productive and efficient - Good health improves learning and skill development-Reduces absenteeism and medical costs -Example – healthier workers in Kerala earn more and contribute to economy.

21. Research helps identify new methods and technologies - Improves skills and productivity of workers -Supports innovation in health and education sectors-Provides data for better decision-making-Example – research in Kerala improves hospital services and teaching quality.

22. Educated migration reduces local job seekers in Kerala-Creates brain drain, limiting skilled workforce locally-Can reduce unemployment pressure temporarily- Leads to higher remittances improving household income-Impacts labor market planning and education-job match in Kerala.

23. Public healthcare – government funded, low cost, serves all, focus on primary and preventive care- Private healthcare – privately funded, higher cost, focus on personalized and specialized care- Public facilities face overcrowding, private facilities offer shorter waiting time- Both complement each other in Kerala's health system.

24 Assess change in disease incidence before and after intervention- Example – immunization programs reducing child morbidity in Kerala -Check coverage and participation of target population-Evaluate improvement in health outcomes and reduced hospital visits-Identify challenges and gaps for further policy action.

25. Social reformers-Examples – Sree Narayana Guru, Ayyankali, and Chattampi Swamikal-Social reformers spread awareness about literacy and equality- Reduced social barriers to education- Promoted education for all, including women and lower castes- Reduced social barriers to education-Founded schools and colleges in Kerala.

Part-C

26. Investment in public health improves population health and life expectancy-Healthy people are more productive and contribute to the economy - Reduces medical expenses for poor

households, promoting financial inclusion-Strengthens human capital, supporting skilled workforce development- Promotes equity by providing access to healthcare for all, including marginalized groups- Reduces disease burden, improving school attendance and workforce participation- Example – Kerala's NRHM and local health initiatives reduced infant mortality to 7 per 1,000 (NFHS-5)-Supports sustainable development goals by improving health, education, and economic growth together.

27. Education improves knowledge, skills, and productivity of individuals-Leads to higher earnings and better job opportunities-Promotes social awareness and participation in civic life-Health investment improves physical and mental well-being- Healthy people are more productive and active in work and learning-Reduces disease burden and medical costs, especially for poor families-Both together strengthen human capital, enhancing economic growth and development- Example – In Kerala, high literacy rate (96%) and good health outcomes support skilled workforce and low infant mortality (7 per 1,000 live births, NFHS-5)-Education benefits are mainly private and social, health benefits impact productivity and longevity.

28. Moral hazard occurs when insured people overuse healthcare services-Government policies reduce this risk by regulating health insurance plans- Introduces co-payments, deductibles, and coverage limits to control overuse-Promotes preventive care programs to reduce long-term claims- Monitors insurance companies to ensure fair pricing and claims-Encourages awareness among insured individuals about responsible use of services- Example – India's Ayushman Bharat PM-JAY includes checks to prevent over-treatment-Reduces unnecessary expenses, improves financial sustainability, and ensures better access to essential health services-Supports equity and efficiency in the health sector.

29. Local governments in Kerala implement primary healthcare and sanitation programs-Manage PHCs, Sub-Centres, waste disposal, and water supply-Promote community health awareness and preventive measures-Example – Kerala Panchayats successfully implement clean Kerala campaign and vector control programs- Effective as grassroots implementers due to local knowledge and community participation-Limitations include fund constraints, staff shortages, and coordination issues-Political interference and weak monitoring reduce efficiency-Despite challenges, local governments improve access, reduce disease burden, and support public health policies in rural areas- Their role is crucial for inclusive and sustainable health development.

30. NHM aims to improve rural health services and infrastructure across India- Strengthened Sub-Centres, PHCs, and CHCs in rural areas-Promoted maternal and child health programs, reducing IMR and MMR-Example – Kerala's rural infant mortality reduced to 7 per 1,000 live births (NFHS-5)-Introduced ASHA workers for community health outreach- Improved availability of medicines, diagnostic facilities, and trained staff-Challenges remain in remote areas, staff shortages, and facility maintenance- Overall, NHM significantly enhanced access, quality, and equity of rural health services-Supports inclusive health development and reduces regional health disparities.

31. Government spending on education focuses on access, quality, and equity-Key factors include literacy goals, human capital formation, and inclusive growth- Main areas of spending – school infrastructure, teacher salaries, training, and scholarships-Investment in Mid-Day Meal Scheme improves student attendance and nutrition- Funding for higher education, research, and vocational training promotes skill development-Example – Kerala spends about 6% of its state budget on education- Central schemes like Samagra Shiksha Abhiyan and NEP 2020 reforms support education quality. Aim – build an educated, skilled, and employable population for sustainable development.

32. Teacher-student ratio means number of students per teacher-A low ratio allows more attention to each student-Improves classroom interaction, learning quality, and academic performance-A high ratio leads to overcrowding and less personal attention- Impacts student motivation, participation, and understanding- Better ratio helps achieve equity and quality education, key goal of NEP 2020-Thus, it directly influences learning outcomes and human capital formation.

33. NEP 2020 aims to provide inclusive and equitable education for all- Focus on Early Childhood Care, universal access, and foundational literacy. Promotes flexible curriculum and multilingual education to reduce social barriers- Targets dropout reduction through open schooling and multiple entry–exit options- Encourages digital learning, but digital divide limits rural participation- Supports gender inclusion through Gender Inclusion Fund- Yet, funding gaps and implementation inequality between states remain- Kerala already performs well, but poorer states lag behind-NEP 2020 is a progressive step, but needs strong public investment for true equality- Hence, policy is promising but not fully effective in reducing inequality.

34. Kerala Model shows high human development with low per capita income-Strong focus on education, health, and social welfare-Achieved literacy rate of 96.2% (Census 2021) —

highest in India-Life expectancy about 75 years, similar to developed countries- Success due to public investment and social reforms by leaders and movements-Promoted gender equality, public health, and land reforms- Criticism: High unemployment, migration dependency, and fiscal stress-Economic growth slower than social development-Environmental and ageing population issues rising- Overall, Kerala Model is socially progressive but economically imbalanced.

35. Healthcare financing means mobilising and using funds for health services-In Kerala, it helps to establish and maintain quality public health infrastructure-Supports primary health centres, hospitals and Medical Colleges, medical research and preventive care-Ensures equitable access to health services for all citizens-State government funds, NHM, and insurance schemes like Karunya Arogya Suraksha Padhathi are major sources-Helps to reduce out-of-pocket expenditure- Enables universal health coverage and disease control programs-Important for sustainability of Kerala's high health indicators-Encourages public-private partnerships for better service delivery.